جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فاطمه سعد حسن محمود
المحافظة : القاهره **الحي / المدينة :** مصر القديمه **العنوان :** ..
رقم البطاقة : 27805170100123 **رقم التليفون :** 01158024631 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 17-05-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه سعد حسن محمود
National ID: 27805170100123

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bone scintigraphy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



National ID – Front
[[DOCTYPE:ID_FRONT]]



National ID – Back
[[DOCTYPE:ID_BACK]]

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	فاطمه سعد حسن محمود	**Case No.**	**385294**
Sex/Age	**Female 48 Y**	**Patient No.**	**29095**
Registered Date	**17/9/2026 11:54:36 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**13/10/2026 01:33:49 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**15/10/2026 09:49:49 AM**	**Pathology No.**	**3014/26**

Pathology Report

Clinical Data: Left breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 17/09/2026, at 11:54 pm. Cold ischemia time is 20 seconds.

Gross Examination:
Two specimens were received:
(1) One grey, white tissue cores, and fragments ranging in size between 0.1 cm & 1.2 cm long, totally submitted.
(2) One tissue core, brownish measured 1.5cm cores, totaily submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
(1) Histologic type: Invasive duct carcinoma
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1)
- **DCIS:** Absent.
- **Microcalcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** Mild.
- **Additional pathological Findings:** Mild minimal necrosis.

(2) Axillary tissue core with subcapsular: Isolated tumor cells (Already examined in serial cells)

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 40% of tumor cells (score 7/8).
- Her2: Strong membrane staining in more than 10% of the tumor cells (score 3+).
- Ki67%: 20%.

Conclusion:

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسى انترناشونال – مصر معمل الباثولوجى
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Reporting Date	**15/10/2026 09:49:49 AM**	**Pathology No.**	**3014/26**

(1) Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Ki67%: 20%.

(2) Left axillary lymph node, Tru-cut biopsy:
Subcapsular isolated tumor cells.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim, MD

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:MAMMOGRAM]]
Mammogram

جوستاف روسي انترناشونال مصر

Patient's Name: **Fatima Saad Hassan Mahmoud**

Date: **2026-09-08**

Referred by: Prof.Dr. **no doctor**

Hospital: **Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 46-year-old female with negative family history, complaining of left breast hardness and skin oedema .

Findings:

Right breast.

- Scattered glandular breast parenchyma. (ACRb)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications
- **Ultrasound** demonstrated homogenous breast parenchyma with no masses or cysts .
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense. (ACR c)
- LIQ area of asymmetry with no spiculations msses or architectural distortion .
- Large breast breast with thickened skin , coarsened trabeculations.
- **Ultrasound** demonstrated central and outer breast 6-7 o'clock irregular heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and moderate internal vascularity on color Doppler study measuring about 3.2x2.2 cm zone c 15 cm from the nipple another smaller7 o'clock similar lesion seen anterior to the index mass about 12 cm from the nipple and 1.4 cm from the skin .
- Thickened skin reaching 1.4 cm with coarsened trabecular and minimal subdermal edema.
- Multiple pathological(more than 3) axillary LNs are noted at level 1, 2 the largest measuring 1.4x1.3 globular in shape hypoechoic with muffled hilum .

Conclusion:-

- ***Right breast (BIRADS 1): normal***
- ***Left breast (BIRADS 5): Locally advanced highly suspicious process with ipsilateral pathological level 1 and 2 axillary Lymph nodes .***

Recommendation:-

- ***For biopsy and histopathological correlation.***

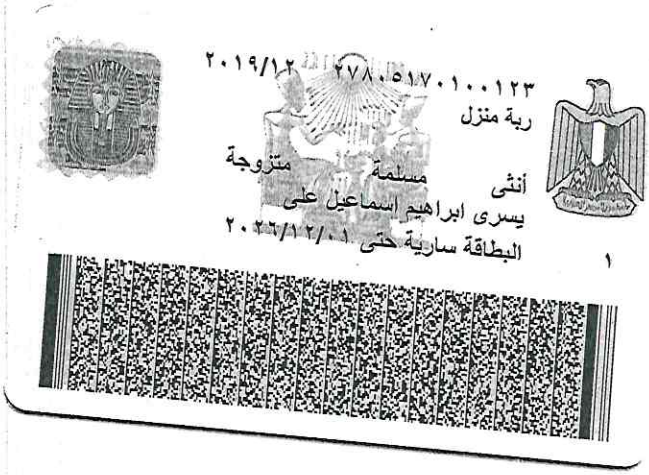
Best Regards,

DR. Dina sayed , M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100% BIRADS 6: Pathologically proven malignant





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Patient's Name:	**Fatima Saad Hassan Mahmoud**
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Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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Mammogram
[[DOCTYPE:MAMMOGRAM]]